

1 Letters created a false impression that members needed to make an  
2 immediate change before the end of Open Enrollment on December 7,  
2018.

3 ¶ 28 The Plan Switch Letters did not include “the following statement  
4 in not less than 8-point type:

5 ‘If you have been receiving care from a health care provider, you may  
6 have a right to keep your provider for a designated time period. Please  
7 contact your HMO’s customer service department, and if you have  
8 further questions, you are encouraged to contact the Department of  
Managed Health Care, which protects HMO consumers, by telephone at  
its toll-free number, 1-888-HMO-2219, or at a TDD number for the deaf  
or hard of hearing at 1-877-688-9891, or online at  
www.hmohelp.ca.gov.’”

9 California Health and Safety Code section 1673.65(f) requires that “[a]  
10 health care service plan and a provider...include [the above language] in  
11 all written, printed, or electronic communications sent to an enrollee that  
12 concern the contract termination or block transfer” of patients.

13 ¶ 29 These Plan Switch Letters to patients were inaccurate and  
14 misleading, and inconsistent with statutory requirements for  
15 communications with Medicare health plan members.

16 ¶ 31 ALLCARE sent the Plan Switch Letters with the intention of  
17 inducing patients covered by CareMore to switch health plans away from  
18 CareMore (and by extension away from CVMG).

19 ¶ 32 ALLCARE intended for the patients receiving the Plan Switch  
20 Letters to rely upon the Plan Switch Letters to alter their choice of health  
21 plan during the open enrollment period for 2018 so as to continue to  
22 receive treatment from their primary care physician. ALLCARE intended  
23 for the patients to have a mistaken impression that they were in imminent  
24 danger of losing their doctors unless they transferred from CareMore to  
25 another HMO.

#### 26 ALLCARE Scheme [to] Destroy CVMG’s Existing Business”

27 ¶ 38 On or about [REDACTED],<sup>2</sup> ALLCARE paid its physicians  
28 their annual contractual performance bonuses for their performance  
[REDACTED]

¶ 39 Beginning on or about February 20, 2019, Defendant ALLCARE,  
through its agents and officers including its Chief Executive Officer,

<sup>2</sup> Portions of the Second Amended Complaint have been Redacted from Public Version. The terms of the Protective Order in the CVMG suit do not permit AllCare’s insurers to have access to sealed documents. However, the redacted portions of the Second Amended Complaint concern only financial details about the physicians’ contracts that are referenced therein. That level of detail in the allegations is unlikely to affect a duty to defend analysis.

Matthew Coury, and its President and Chairman of the Board, Randy Winter, told physicians who were affiliated with both Defendant and CVMG that Defendant would withhold any further annual contractual performance bonuses unless the physicians terminated their CMVG contracts.

¶ 40 ALLCARE dressed up this part of its scheme in colorful language, referring to it as the Preferred Physician Quality Incentive Program (“PPQI”). To be eligible for the PPQI, physicians could not contract with any other IPA in Stanislaus County that holds one or more of the same HMO contracts as ALLCARE. ALLCARE referred to this requirement as the prerequisite (referred to herein as the “Exclusivity Prerequisite”). ALLCARE told physicians that, unless they gave notice of termination of their contract with CVMG within 60 days, they would lose their performance bonuses.

¶ 41. ALLCARE’s goal was for 100% of ALLCARE’s physicians to terminate their contract with CVMG, decimating CVMG’s network and destroying CVMG’s business. In this way, ALLCARE would divert membership from CVMG by making sure that patients had no choice but to see their PCP through ALLCARE.

¶ 42. ALLCARE’s Exclusivity Prerequisite was intended to, and in fact, did, put significant financial pressure on PCPs. [REDACTED]

[REDACTED] This loss of compensation was significant for all of these physicians.

¶ 43. The only IPA in Stanislaus County impacted by this policy was CVMG. This was known to ALLCARE when it developed and announced the Exclusivity Prerequisite. As such, the Exclusivity Prerequisite targeted CVMG only.

¶ 44. Thus, any physician who failed to terminate her contract with CVMG would lose her annual contractual performance bonus [REDACTED].

¶ 46. ... ALLCARE has communicated to its physicians that ALLCARE will penalize non-exclusive CVMG-contracted physicians by refusing to pay the contractually mandated performance bonuses. Non-exclusive CVMG-contracted physicians will not be paid for meeting their performance targets in breach of ALLCARE’s contract with the physicians.

¶ 47. ALLCARE also promised to divert funds earned by physicians (as a result of patients assigned to those physicians through ALLCARE) who remain contracted with CVMG to those physicians who terminate their contracts with CVMG. Thus, the “exclusive” physicians will receive funds earned by the non-exclusive physicians.

¶ 48. ....Physicians were pressured by ALLCARE and its agents to terminate their contracts with CVMG and had a resulting fear that their livelihoods were at risk if they opted not to terminate their contracts with CVMG.

¶ 49. To further its scheme, ALLCARE and/or its agents also disparaged

1 and defamed CVMG by communicating to physicians and medical  
 2 providers that CVMG would no longer be in existence as of July 2019.  
 3 ALLCARE also communicated to physicians and medical providers that  
 4 CVMG did not pay bonuses to physicians in 2018 and that this failure to  
 pay bonuses was evidence that CVMG's business was not doing well.  
 These statements were inaccurate and contrary to the reason for CVMG's  
 decision related to bonuses.

#### 5 ALLCARE Scheme [to] Destroy CVMG's Prospective Business"

6 ¶52. ... ALLCARE has now focused its efforts on destroying the  
 7 upcoming CareMore contract. For instance, as a result of ALLCARE's  
 8 assault on CVMG's PCP Network, CVMG's network recently was  
 9 deemed inadequate to serve CareMore's members (even though it had  
 been determined adequate previously). ALLCARE's improper conduct  
 decimated CVMG's network, directly leading to the "inadequate"  
 designation. [REDACTED]

10 [REDACTED]

11 ¶53. .... It is critical for CVMG's business that it be able to retain and  
 12 attract physicians to its network. Should its network become inadequate,  
 13 it is at risk for losing the CareMore contract, upon which CVMG's  
 14 business now depends. ALLCARE continues to take actions to interfere  
 with CVMG's relationships with physicians and members, in an effort to  
 destroy CVMG's network and its business.

15 ¶54. .... Because CVMG understood that ALLCARE's contracts with  
 16 its PCPs allow those physicians to contract with other IPAs for members  
 17 with health plans with whom ALLCARE was not contracted (e.g. a non-  
 overlapping health plan), CVMG contacted PCPs requesting that they  
 18 contract with CVMG for the CareMore business only.... CVMG sent  
 these physicians correspondence inviting them to contract with CVMG  
 for CareMore only....

19 ¶55. Most physicians declined CVMG's offer because they believed  
 20 that ALLCARE's Exclusivity Prerequisite prevented them from  
 21 contracting with CVMG. In other words, these physicians refused to  
 22 contract with CVMG as a direct result of ALLCARE'S wrongful  
 23 conduct, which led physicians to believe that they would lose their  
 contractual bonuses. Although a few physicians agreed to contract with  
 CVMG for CareMore only, those physicians ultimately backed out of  
 their decision after additional contact from an ALLCARE representative  
 who told them ALLCARE would withhold the bonuses if they contracted  
 with CVMG.

24 ¶56. .... ALLCARE's intentional misrepresentation, deceit and/or  
 25 concealment of the nature of its contract with physicians was intended to  
 26 deprive CareMore and CVMG of members and to otherwise cause injury  
 27 to CareMore and CVMG. ALLCARE knew that such intentional  
 28 misrepresentation, deceit and/or concealment of the nature of its contract  
 with physicians would or was likely to disrupt patient-physician  
 relationships and cause ALLCARE-contracted physicians to lose  
 members. ALLCARE sought to "make up" for any such losses to  
 physicians by redirecting funds earned by CVMG-contracted physicians  
 to the now-exclusive physicians who had terminated their contracts with

1 CVMG.

2 ¶62. .... ALLCARE's actions have prevented those physicians from  
3 retaining their relationship with those patients and prevented those  
4 patients from retaining their relationship with those physicians.  
5 ALLCARE has done this by communicating to physicians that the  
6 Exclusivity Prerequisite prevents physicians from contracting with  
CVMG at all, including for CareMore only. This is contrary to the  
express contractual language in ALLCARE's contracts with physicians.  
ALLCARE's actions will cause unnecessary disruptions in  
patient/physician relationships.

7 ¶63. ALLCARE's scheme is not limited to pressuring the PCPs.  
8 Specifically, ALLCARE has also targeted the patients.....  
9 [C]ommunications were intended to induce the members to terminate  
10 their relationship with CVMG in order to retain their relationship with  
11 their physician. ALLCARE participated in and/or was aware of  
12 communications to members related to the termination of the physician's  
13 relationship with CVMG and intended that such communications would  
14 induce members to terminate their relationship with CVMG in order to  
15 retain access to their physician. In fact, "form" communications from  
16 physicians in various practices to their CareMore members were sent to  
those members as recently as October 2019. ALLCARE was aware of  
and/or participated in these communications to CareMore members.

13 ¶65. ALLCARE's actions have caused, and will continue to cause,  
14 CVMG to suffer significant harm.

15 ¶¶ 66-79 By Loss of PCPs, Loss of Members, Network Adequacy, Loss  
16 of potential HMO contracts, Loss of Existing HMO contracts, Harm to  
the CareMore Contract, and Harm To Healthcare Consumers.

17 ¶84. As a separate and independent wrongful act,<sup>2</sup> Defendant's threat to  
18 refuse to pay earned annual contractual performance bonuses to  
19 physicians in the future unless they terminate their contracts with  
20 CVMG, while still demanding but not paying for the same performance,  
21 is a breach of Defendant's contract(s) with those physicians, a breach of  
22 the implied covenant of good faith and fair dealing inherent in that  
23 contract, and/or violation of California law (including placing an undue  
24 burden on trade).<sup>3</sup> ALLCARE's conduct is coercive in light of the actual  
25 and/or perceived severe harm those physicians would suffer if they  
26 refused ALLCARE's demand, including but not limited to impacts to  
27 their livelihood and personal and business finances, impacts to the  
28 livelihoods of their staff, and the professional cost involved in severing  
relationships with patients who are contracted with CVMG. As alleged  
above, ALLCARE further interfered with CVMG's contracts and  
relationships by making defamatory statements about CVMG to  
physicians.

ALLCARE and/or its agents also disparaged and defamed CVMG by  
communicating to physicians that CVMG would no longer be in  
existence as of July 2019.

¶96. As a separate and independent wrongful act, Defendant's threat to  
refuse to pay earned annual contractual performance bonuses to  
physicians in the future unless they terminate their contracts with

CVMG, while still demanding but not paying for the same performance, is an illegal restraint in trade, a breach of Defendant's contract(s) with those physicians and/or a breach of the implied covenant of good faith and fair dealing inherent in that contract. ALLCARE's conduct is coercive in light of the actual and/or perceived severe harm those physicians would suffer if they refused ALLCARE's demand, including but not limited to impacts to their livelihood and personal and business finances, impacts to the livelihoods of their staff, and the professional cost involved in severing relationships with patients who are contracted with CVMG. As alleged above, ALLCARE further interfered with CVMG's contracts and relationships by making defamatory statements about CVMG to physicians.

¶122. As a separate and independent wrongful act, Defendant's threat to refuse to pay earned annual contractual performance bonuses to physicians in the future unless they terminate their contracts with CVMG, while still demanding but not paying for the same performance, is an unlawful restraint in trade, a breach of Defendant's contract(s) with those physicians and/or a breach of the implied covenant of good faith and fair dealing inherent in that contract. ALLCARE's conduct is coercive in light of the actual and/or perceived severe harm those physicians would suffer if they refused ALLCARE's demand, including but not limited to impacts to their livelihood and personal and business finances, impacts to the livelihoods of their staff, and the professional cost involved in severing relationships with patients who are contracted with CVMG. As alleged above, ALLCARE further interfered with CVMG's contracts and relationships by making defamatory statements about CVMG to physicians.

¶199. Here, ALLCARE's Exclusivity Prerequisite prevents physicians from freely engaging in their lawful profession as physicians. ALLCARE's policy penalizes physicians who contract with CVMG, refusing to pay those physicians their contractual performance bonuses unless they terminate their contracts with CVMG. The contractual performance bonus owed by ALLCARE to these physicians

ALLCARE's policy functions as a penalty for physicians who choose to contract with CVMG and withholds compensation from those physicians.

¶200. Furthermore, ALLCARE's threats to withhold contractual performance bonuses from physicians who contract with CVMG has prevented physicians who desire to contract with CVMG from entering into and/or maintaining those contracts.

¶201. These threats restrain physicians from "engaging in [their] lawful profession, trade, or business" in contravention of Section 16600.

¶206. As a direct and proximate result of Defendant's conduct as alleged herein, CVMG has lost all its existing contracts with health plans.

**[Exhibit "5"]**.